Session 7 (PROTECT & WRITE: ethics, integrity, Methods and Results): Exercise sheet

Foundations of Clinical Research · Level 1 · Neudata · *#ClearDataClearImpact* · Week 7 · Saturday 12 December 2026 · online, 19:30–22:30

Name: _____ **Department:** _____

Type your answers into this Word file or print the PDF.

Part	Task	When
A	Integrity scenarios: green, amber or red?	End of section 7.2, finish at home
B	Proposal builder 7: write Part 4 of your proposal (ethics, registration, feasibility, publication plan)	Before the break, finish at home
C	**Manuscript workshop 1**: write the Results section for your FLUID-ICU question	After section 7.3, finish at home

Files you need (sent by email): the results pack for your question (`Results_Pack_QN.pdf`), the manuscript template (`Manuscript_Template_IMRaD.docx`), Paper 1 (Boulet 2024) and Paper 2 (Hyun 2023).

Part A: Integrity scenarios: green, amber or red?

Task. For each scenario, choose a colour and write one sentence explaining why. In class, do scenarios 1, 3 and 4 alone, then vote in the poll and be ready to give your reason when the instructor asks. Do scenarios 2, 5 and 6 at home.

> **Choose from: GREEN** = acceptable as described · **AMBER** = acceptable **only if** something is fixed or disclosed (say what) · **RED** = research misconduct or a serious breach. Do not do it.

#	Scenario	Colour	Why?
1	Three patients' day-3 fluid charts are lost. A resident "remembers roughly" what their balances were and enters values so the dataset is complete.		
2	After seeing the results, the team removes two patients with "strange" fluid values because they weaken the association. The paper does not mention it.		
3	The head of department did not take part in the study but asks to be last author "because it is my department".		
4	(a) A nurse uses an AI chatbot to improve the English of the Discussion she wrote, checks every sentence, and states this in the Acknowledgements. (b) A colleague uploads the study spreadsheet (names and hospital numbers included) to a free public chatbot and asks it to "run the statistics".	(a)	
		(b)	
5	A cohort of patients with sepsis like FLUID-ICU is published as three papers (mortality, acute kidney injury and ICU stay) in three journals, none mentioning the others.		
6	Two paragraphs of a published review are copied word for word into your Introduction. The review is cited at the end of the paragraph, but there are no quotation marks.		

Part B: Proposal builder 7: ethics, registration, feasibility and publication plan

Task. Fill in this worksheet for **your own** proposal. In class, the instructor first fills in each block for the running example (a retrospective cohort of fluid balance and 28-day mortality in adults with septic shock in our ICU). After each block, write your own answer alone and be ready to read it out when the instructor asks. Finish at home.

Your answers become Part 4 of your written proposal and slides 5 (ethics and feasibility) and 6 (publication plan) of your presentation. Email the written proposal and the 6 slides before 19:30 on Saturday 19 December 2026, Session 8 (`Proposal_Brief.pdf`).

B1. Ethics and consent

Question	Your answer
Which ethics committee will review the study? What type of review do you expect (exempt / expedited / full)? The committee decides.	
Who are the participants, and can they decide for themselves when they become eligible?	
Who gives consent (patient / legal representative / deferred consent / waiver requested)? Use the ICU consent flowchart (Session 7 slides).	
Who takes consent, when and where?	
What are the risks and burdens for participants, and how are they minimised?	
Does the study need registration? Trials: yes, before the first patient, in ClinicalTrials.gov or another WHO registry. Systematic reviews: PROSPERO. Observational studies: recommended.	

B2. Confidentiality

Question	Your answer
Which direct identifiers will you collect (if any), and why?	
How will each patient get a study code?	
Where is the key file (code to identity) kept, and who can open it?	
Where will the de-identified data be stored (institution-approved, password-protected)?	

B3. Feasibility check (tick and comment)

✓	Question	Comment
☐	**Time:** realistic timeline (ethics 1–3 months, recruitment, analysis, writing)?	
☐	**Money:** budget headings named (staff time, materials/tests, publication fee)?	
☐	**People:** who collects data (nights, weekends), who analyses, protected time?	
☐	**Patients:** how many eligible patients did your unit see last year? How many can you realistically enrol?	
☐	**Data:** are the key variables recorded routinely? How much is likely missing?	

Biggest feasibility risk: _____ One fix: _____

B4. Publication plan (slide 6 of your presentation)

Item	Your answer
Working title (from your PICO: design, I vs C, P, O, in 10 to 16 words)	
Target journal (and a back-up), checked in DOAJ / NLM Catalog, with fees and licence known	
Reporting guideline (CONSORT / STROBE / STARD / PRISMA / SPIRIT / CARE)	
Registration needed? Where? (from B1)	
Authors (all four ICMJE criteria) and their roles (CRediT)	
Conflicts of interest and funding to declare	
Data-sharing statement (what, where, under which conditions)	
Conference where you would present an abstract first	

Structured-abstract skeleton. One line each. Results are the results you *expect to report* (format, not numbers).

Section	One line
Background	
Methods	
Results (expected)	e.g. "28-day mortality: n/N per group, RR (95% CI)"
Conclusion	Write the sentence you would use if the result is positive, and the one you would use if it is not.

Part C: Manuscript workshop 1: the Results section

Task. Write the **Results section** for your FLUID-ICU question, using only the results pack for that question. In class we first write the Results for Q1 together, block by block: for each block you write the sentences alone, then share them when the instructor asks. Then start the Results for your own question. Finish at home and hand it in (see "Deliverable" below). Only the Results section is required. You may add other sections in the same file (`Manuscript_Brief.pdf`).

Question	Topic	Results pack
Q1	Positive fluid balance by day 3 and 28-day mortality	`Results_Pack_Q1.pdf`
Q2	Cumulative fluid balance and acute kidney injury (AKI)	`Results_Pack_Q2.pdf`
Q3	Cumulative fluid balance and ventilator-free days	`Results_Pack_Q3.pdf`
Q4	Initial lactate and 28-day mortality, complete case vs multiple imputation	`Results_Pack_Q4.pdf`
Q5	Fluid balance and ICU length of stay among survivors	`Results_Pack_Q5.pdf`

The class draft is sent as a PDF after the session. If you chose Q1, write it in your own words.

Results checklist (STROBE items)

- ☐ Participants first: numbers enrolled, analysed and excluded, with reasons (13)
- ☐ Table 1 in one or two sentences, with numbers. Missing data reported (14)
- ☐ Outcome numbers per group, with denominators (15)
- ☐ Main result: **crude and adjusted** estimate with 95% CI (16)
- ☐ Other analyses in your pack: sensitivity, imputation or subgroup (17)
- ☐ Table 1 and one main table or figure that stand alone
- ☐ No interpretation ("because", "suggests") and no causal words
- ☐ A working title and the AI-use statement

Question-specific reminders

- **Q3:** ventilator-free days are skewed and count death as zero.
- **Q4:** report both the complete-case and the multiple-imputation estimates.
- **Q5:** survivors only. Use medians and IQR.

Methods (taught in 7.3, optional to submit)

If you also submit the Methods, use STROBE items 4 to 12: design and setting, participants, variables, data sources, bias, study size ("all eligible patients in the cohort were included"), statistical methods and missing data. The facts are in your results pack, `SAP_FLUID-ICU.pdf` and `Data_Dictionary.pdf`.

Deliverable

Your Results section, named `Surname_Firstname_Results_QN.docx`, emailed to your reviewer and to the instructor (myluong1710@gmail.com) **by 23:59 on Sunday 20 December 2026 (Vietnam time)**. Your reviewer is announced at the end of today's session.